

D4 — H&P — PGY-1

Dr. J. Martinez, PGY-1 | Medicine | 02/18/2026 07:45

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

HISTORY AND PHYSICAL — ADMISSION NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156
Date: 02/18/2026 07:45 | Provider: Dr. J. Martinez, PGY-1 | Attending: Dr. M. Torres

CHIEF COMPLAINT: COPD exacerbation

HISTORY OF PRESENT ILLNESS:

74 y/o male with extensive PMH (COPD, HFrEF EF 30%, afib on apixaban, HTN, T2DM, CKD 3b, MDD, chronic back pain, GERD, hyperlipidemia, BPH, hypothyroidism, gout, chronic constipation) who presented to the ED with 3 days of worsening dyspnea, productive cough with yellow-green sputum, low-grade fever, and wheezing. ED workup significant for SpO2 89% on RA, WBC 13.2, BNP 480, CXR showing hyperinflation without consolidation. Received nebs, IV steroids, and 1L NS in ED. Admitted for acute COPD exacerbation.

PAST MEDICAL HISTORY: As above.
PAST SURGICAL HISTORY: Appendectomy (1988), Right knee arthroscopy (2010)
ALLERGIES: Sulfa drugs – rash

HOME MEDICATIONS (reconciled from prior discharge summary + patient report + scanned family list):

1. Apixaban 5 mg PO BID – afib
2. Carvedilol 25 mg PO BID – CHF/rate control
3. Lisinopril 20 mg PO daily – CHF/HTN
4. Furosemide 40 mg PO daily – CHF
5. Spironolactone 25 mg PO daily – CHF
6. Amlodipine 5 mg PO daily – HTN
7. Metformin 1000 mg PO BID – T2DM
8. Glipizide 5 mg PO BID – T2DM
9. Tiotropium (Spiriva) 18 mcg inhaled daily – COPD
10. Fluticasone/salmeterol (Advair) 250/50 inhaled BID – COPD
11. Albuterol MDI 2 puffs Q4-6H PRN – COPD
12. Sertraline 100 mg PO daily – MDD
13. Omeprazole 20 mg PO daily – GERD
14. Docusate sodium 100 mg PO BID – constipation
15. Senna 8.6 mg PO QHS PRN – constipation
16. Atorvastatin 40 mg PO daily – hyperlipidemia

17. Levothyroxine 75 mcg PO daily – hypothyroidism
18. Tamsulosin 0.4 mg PO daily – BPH
19. Allopurinol 100 mg PO daily – gout
20. Vitamin D3 2000 IU PO daily
21. Acetaminophen 500 mg PO Q6H PRN – pain
22. Ibuprofen 400 mg PO Q8H PRN – pain
23. Cyclobenzaprine 10 mg PO QHS PRN – muscle spasms
24. CBD oil ~25 mg PO daily – pain (OTC supplement)
25. Lidocaine 5% patch 1 daily (12h on/12h off) – pain (per recent PCP note)
26. Capsaicin 0.075% cream TID PRN – pain (per recent PCP note)

PHYSICAL EXAMINATION:

Vitals: HR 92, BP 142/78, RR 20, Temp 99.8°F, SpO2 94% on 2L NC, Wt 82 kg
General: Elderly male, comfortable on 2L NC, speaking in full sentences this AM
HEENT: MMM, no JVD appreciated (difficult to assess with habitus)
Lungs: Diffuse bilateral expiratory wheezing, decreased air movement at bases bilaterally
CV: Irregularly irregular, soft S1/S2, no S3 or S4 appreciated, no murmurs
Abdomen: Soft, NT, ND, normoactive bowel sounds
Ext: Bilateral LE edema 1+ (per patient, baseline), no calf tenderness
Neuro: A&Ox4, no focal deficits

LABS (AM draw):

Na 136 | K 4.2 | Cl 101 | CO2 28 | BUN 32 | Cr 1.6 | Glucose 186
WBC 13.2 | Hgb 11.2 | Plt 198
BNP 480

ASSESSMENT & PLAN:

1. Acute COPD exacerbation – infectious trigger likely given productive cough + low-grade fever
 - Start methylprednisolone 60 mg IV Q8H
 - Nebulized albuterol 2.5 mg Q4H
 - Nebulized ipratropium 0.5 mg Q6H
 - Start azithromycin 500 mg IV x1, then 250 mg PO daily x 4 days
 - HOLD home inhalers while on scheduled nebulizers
 - Supplemental O2 to maintain SpO2 > 90%
2. CHF (HFrEF, EF 30%) – appears at baseline. BNP 480 likely chronic.
 - Continue carvedilol, lisinopril, spironolactone, amlodipine
 - Continue home furosemide 40 mg PO daily
 - Monitor I/Os and daily weights
3. Atrial fibrillation – rate controlled
 - Continue apixaban 5 mg BID
 - Continue carvedilol for rate control

4. T2DM

- HOLD metformin (acute illness, possible imaging with IV contrast)
- Continue glipizide 5 mg BID
- Fingerstick glucose QAC and QHS

5. CKD 3b – Cr 1.6, near baseline

- Monitor BMP daily
- Renally dose medications appropriately

6. MDD

- DISCONTINUE sertraline – concern for hyponatremia in setting of acute illness with IV fluids and potential aggressive diuresis if CHF flares. Will reassess.

7. Chronic back pain

- Continue acetaminophen PRN
- HOLD ibuprofen (NSAID caution with CKD + CHF + acute illness)
- Continue cyclobenzaprine QHS PRN
- Continue lidocaine patch, capsaicin cream

8. GERD – continue omeprazole

9. Constipation – continue docusate, senna PRN

10. Hypothyroidism – continue levothyroxine

11. Gout – continue allopurinol

12. BPH – continue tamsulosin

13. Hyperlipidemia – continue atorvastatin

Electronically signed by: Dr. J. Martinez, MD (PGY-1)

Cosigned by: Dr. M. Torres, MD (Attending)

Date: 02/18/2026 08:30